

STRENGTHENING CONTRACEPTIVE ACCESS AND COMMODITY SECURITY FOR WOMEN AND ADOLESCENT GIRLS



EXECUTIVE SUMMARY

Access to modern family planning is a fundamental component of safety, reproductive healthcare and an essential investment in the health, education and economic potential of women, adolescent girls and communities.

Despite these essential benefits, many women, adolescent and vulnerable populations still struggle to access quality services, facing youth-friendly barriers, stock-outs of essential commodities and widespread stigma.

County governments play a critical role in addressing these challenges through evidence-based policies, strategic investments and multi-sectoral partnerships.

Strengthening contraceptive access contributes directly to improved maternal and child health, reduced adolescent pregnancy, empowered women and sustainable county development.

WHY THIS MATTERS

Every woman and adolescent girl should be able to make informed decisions about whether and when to have children. When contraceptive access is strengthened:

- ✓ Fewer unintended pregnancies
- ✓ Reduced maternal and newborn deaths
- ✓ Improved educational outcomes
- ✓ Increased economic productivity
- ✓ Stronger families and communities
- ✓ Reduced pressure on health facilities
- ✓ Enhanced adolescent opportunities

Consequently, counties benefit from healthier populations, smaller class sizes, reduced demand on social services, and overall sustainable county development.

CURRENT COUNTY CHALLENGES

- 1 Frequent contraceptive stock-outs.
- 2 Limited funding for family planning commodities.
- 3 Weak training for healthcare workers.
- 4 Inadequate youth-friendly service delivery.
- 5 Limited provider training on adolescent responsive care.
- 6 Gaps in data, systems and accountability.
- 7 Sociocultural myths and stigma affecting uptake.
- 8 Limited community awareness about sexual and reproductive health rights.



PRIORITY COUNTY ACTIONS



Strengthen Financing

- Establish dedicated budget lines for family planning commodities and services in the County Integrated Plans (CIPs) and Annual Development Plans (ADPs)



Strengthen Commodity Security

- Improve forecasting and quantification
- Strengthen procurement and last mile distribution
- Implement real-time stock monitoring



Expand Youth-Friendly Services

- Establish adolescent-friendly service corners
- Train healthcare workers on youth-responsive care
- Extend service hours where appropriate
- Train providers on adolescent responsive care



Strengthen Provider Capacity

- Train healthcare workers on:
- Rights-based family planning
 - Confidentiality
 - Counselling skills
 - Adolescent responsive care
 - Non-discriminatory service communication



Increase Community Awareness

- Support community mobilisation to:
- Address myths and misconceptions
 - Promote positive attitudes
 - Engage community leaders
 - Improve knowledge about rights and community health by supporting reproductive health



Strengthen Monitoring and Accountability

- Counties should routinely monitor:
- Availability of commodities and services
 - Stock-out rates
 - Provider training
 - Youth service uptake
 - Client satisfaction and feedback
 - Data use for planning and decision-making

WHAT KENYAN LAW AND POLICY SAY

LAW	WHAT KENYAN LAW AND POLICY PROVIDE	IMPLICATIONS FOR COUNTY GOVERNMENTS
Constitution of Kenya	Article 43 on the Constitution guarantees every person the right to the highest attainable standard of health, including reproductive healthcare, and the right to information and education.	Counties should ensure family planning information and services are available and accessible to all.
Access to Family Planning Act	National legislation facilitates access to voluntary family planning services for all persons, including adolescents.	Counties should invest in youth-friendly services and eliminate barriers.
Health Act	The National Reproductive Health policy guides the provision of quality reproductive health services, including family planning.	Counties should prioritize youth-friendly services, train providers, and ensure consistent commodity availability.
Reproductive Healthcare Policy	The National ASRH Policy prioritizes making adolescent reproductive health services accessible and comprehensive.	Counties should integrate adolescent reproductive health into all county reproductive health programmes.
Contraceptive Security in Health Facilities	National policies ensure continuous access to contraceptives by maintaining strong commodity security and supply systems.	Counties should improve procurement systems & stock management, and maintain uninterrupted supplies.
Community Health Strategy	National strategy recognizes the critical role of community structures in increasing awareness and enhancing service uptake.	Counties should strengthen community health systems & collaborate with community organizations.
Non-Discrimination Principle	National policies prohibit discrimination, including on grounds of age, marital status, disability or socio-economic status.	Counties should establish inclusive services that address the needs of girls, women and all vulnerable populations.

COMMON MYTHS vs. FACTS

Myth	Fact
Contraceptives encourage adolescents to have sex.	Evidence shows that access to accurate information and services helps make informed choices and reduces unintended pregnancies among young people.
Family planning is unimportant, just use traditional methods.	Family planning is comprehensive and empowering, and includes modern methods that are safe, effective and evidence-based.
Only married women should use family planning.	Family planning is a right for all, regardless of age or marital status. Everyone deserves access to quality services.



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COUNTY OVERSIGHT CHECKLIST

Oversight Question	Yes	No
Is there a dedicated county budget line for family planning commodities?	☐	☐
Are contraceptive commodities consistently available across county facilities?	☐	☐
Are youth-friendly services available in public health facilities?	☐	☐
Have healthcare workers been trained on adolescent-responsive family planning?	☐	☐
Are commodity stock-outs monitored and reported?	☐	☐
Are community awareness programmes addressing myths and stigma?	☐	☐
Are adolescents and community representatives involved in planning and decision-making?	☐	☐

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